



Oral Cavity & Salivary Glands

Pathology GIT Block · MEDucation by Mattin Majid · Practice MCQs

HOW TO USE THIS SET

20 questions, mixed difficulty, 10 case-based. Attempt every question before checking the answer key on the last page.

● 4 Easy

● 10 Medium

● 6 Hard

1.

EASY

Inflammation of the gum is termed:

- A. Gingivitis
- B. Cheilitis
- C. Sialadenitis
- D. Glossitis
- E. Stomatitis

2.

EASY

Approximately what proportion of oral cavity cancers are squamous cell carcinoma?

- A. 50%
- B. 65%
- C. 80%
- D. 95%
- E. 99%

3.

EASY

The commonest lesion of the minor salivary glands is:

- A. Oncocytoma
- B. Basal cell adenoma
- C. Mucocele
- D. Adenoid cystic carcinoma
- E. Warthin tumour

4.

EASY

The commonest benign salivary gland tumour is:

- A. Pleomorphic adenoma
- B. Warthin tumour
- C. Basal cell adenoma
- D. Oncocytoma
- E. Cystadenoma

5.

MEDIUM

CASE

A 30-year-old woman develops grouped 1–3 mm vesicles on her lower lip after a day in strong sunlight. They resolve within 8 days. Smears show glassy intranuclear inclusions.

Which mechanism best explains this episode?

- A. Autoimmune ulceration of unknown cause
- B. Reactivation of latent HSV-1
- C. HPV-driven epithelial proliferation
- D. Primary EBV infection
- E. Blocked minor salivary gland duct

6.

MEDIUM CASE

A 25-year-old has recurrent, painful oral ulcers, each covered by a thin fibrinopurulent exudate with a surrounding erythematous halo, healing in 7–10 days.

Which systemic disease is listed as an association of this lesion?

- A. Coeliac disease
- B. Rheumatic heart disease
- C. Chronic kidney disease
- D. Hepatitis B
- E. Sarcoidosis

7.

MEDIUM CASE

A 27-year-old pregnant woman has a rapidly growing, richly vascular, ulcerated nodule on the gingiva.

What is the most likely diagnosis?

- A. Pleomorphic adenoma
- B. Squamous cell carcinoma
- C. Pyogenic granuloma
- D. Mucocele
- E. Fibroma

8.

MEDIUM CASE

A 60-year-old has a white plaque on the buccal mucosa that cannot be scraped off. After work-up the plaque is attributed to lichen planus.

Which statement is correct?

- A. It is erythroplakia
- B. It is candidiasis
- C. It is not leukoplakia, because leukoplakia cannot be explained by another defined disease
- D. It is leukoplakia, because it cannot be scraped off
- E. It is leukoplakia with a >50% malignant risk

9.

MEDIUM

Approximately what is the malignant transformation risk of erythroplakia?

- A. <5%
- B. 5–10%
- C. 10–25%
- D. 25–50%
- E. >50%

10.

MEDIUM CASE

A 58-year-old man both smokes heavily and drinks alcohol daily.

Which statement best describes his risk of oral SCC?

- A. Alcohol is the best-established risk factor and tobacco adds nothing
- B. Alcohol has no influence once tobacco is accounted for
- C. Risk is the same as for a non-smoker who drinks
- D. The two habits interact to greatly increase risk beyond either alone
- E. Risk is determined by HPV status alone

11.

MEDIUM

A biopsy-proven oral SCC of the lateral tongue is being staged. Which is the commonest regional site of metastasis?

- A. Lung
- B. Liver
- C. Cervical lymph nodes
- D. Mediastinal lymph nodes
- E. Inguinal lymph nodes

12.

MEDIUM CASE

A dehydrated elderly patient has a painful, swollen salivary gland with an obstructing duct stone.

Which organisms most likely cause this sialadenitis?

- A. HPV 16 and 18
- B. Mumps virus
- C. HSV-1
- D. Staphylococci or streptococci
- E. Candida

13.

MEDIUM

Which statement distinguishes an extravasation mucocele from a retention mucocele?

- A. Retention mucoceles occur only in children
- B. Extravasation follows rupture of a duct with saliva leaking into the surrounding stroma
- C. Retention mucoceles are malignant
- D. Extravasation occurs only in the parotid gland
- E. Extravasation follows blockage of a duct

14.

MEDIUM CASE

Histology of a salivary gland tumour shows variable mixtures of squamous cells, mucus-secreting cells and intermediate cells.

Which statement about this tumour is correct?

- A. It arises from a pleomorphic adenoma
- B. It is characteristically well demarcated and painless
- C. It is defined by ductal and myoepithelial cells in a chondroid matrix
- D. It is the commonest benign salivary tumour
- E. It is the commonest primary malignant salivary gland tumour

15.

HARD CASE

A 55-year-old is found to have a tumour in a minor salivary gland of the palate. A colleague reassures him that "salivary gland tumours are nearly always benign".

Which is the best response?

- A. That holds best for the parotid (80% benign); in the smallest glands only about 40% are benign
- B. That is correct for every salivary gland
- C. Malignancy is more likely in the parotid than in minor glands
- D. Minor gland tumours are almost always pleomorphic adenomas
- E. Gland size has no bearing on malignancy

16.

HARD CASE

A woman had a parotid pleomorphic adenoma shelled out with an incompletely excised margin.

Which consequences are associated with this tumour?

- A. Early distant metastasis in every case
- B. Local recurrence, and carcinoma can arise within it
- C. Transformation into Warthin tumour
- D. Conversion into mucoepidermoid carcinoma
- E. Spontaneous regression in most cases

17.

HARD

Which pairing of salivary tumour and its rank in the slide table is correct?

- A. Basal cell adenoma — 15% of tumours
- B. Oncocytoma — commonest benign tumour
- C. Adenoid cystic carcinoma — second commonest malignant tumour (6%)
- D. Acinic cell carcinoma — commonest malignant tumour
- E. Warthin tumour — second commonest benign tumour (5%)

18.

HARD

CASE

A 66-year-old pipe smoker who drinks daily has an ulcerated, indurated lesion of the lateral tongue.

Biopsy will most likely show:

- A. Nests and islands of malignant keratinocytes invading the stroma
- B. Glassy intranuclear inclusions in squamous cells
- C. A highly vascular proliferation of organising granulation tissue
- D. A mucin-filled cystic space
- E. Ductal and myoepithelial cells in a chondroid matrix

19.

HARD

Which factor is only weakly associated with oral SCC?

- A. Combined alcohol and tobacco use
- B. HPV 16 and 18 (found in 30-50% of cases)
- C. Pipe smoking
- D. Protracted irritation
- E. Smokeless tobacco

20.

HARD

Which is NOT listed among causes of sialadenitis?

- A. Staphylococcal infection after duct obstruction
- B. HPV infection
- C. Trauma
- D. Sjögren syndrome
- E. Mumps

ANSWER KEY & RATIONALE

Don't peek until you've committed to an answer on every question.

Q	Ans	Why
1	A	Gingivitis = gum; cheilitis = lips; glossitis = tongue; stomatitis = oral mucosa.
2	D	About 95% of oral cavity cancers are SCC.
3	C	Mucocele — a mucous-filled cyst from duct blockage (retention) or rupture (extravasation).
4	A	Pleomorphic adenoma makes up about 50% of salivary gland tumours.
5	B	Recurrent herpetic stomatitis: latent HSV-1 reactivated (UV light is a listed trigger); inclusions and 1–3 mm vesicles resolving in 7–10 days.
6	A	Aphthous ulcers are associated with immunologic disorders: coeliac disease, inflammatory bowel disease and Behçet disease.
7	C	Pyogenic granuloma: gingival, vascular organising granulation tissue, seen in children, young adults and in pregnancy.
8	C	Leukoplakia is a white patch that cannot be scraped off and cannot be characterised as any other disease; lichen planus and candidiasis are excluded.
9	E	Erythroplakia: more than 50% (leukoplakia is 3–25%).
10	D	Tobacco is the best-established influence; alcohol is weaker alone, but the two habits interact to greatly increase risk.
11	C	Cervical nodes are the commonest regional site; mediastinal nodes, lungs and liver are frequent sites of distant metastasis.
12	D	Bacterial sialadenitis (staph or strep) follows duct obstruction by stone or food particles, or dehydration. Mumps is the viral cause.
13	B	Retention = duct blockage; extravasation = duct rupture with leakage of saliva into surrounding connective tissue stroma.
14	E	Mucoepidermoid carcinoma — the commonest primary malignant salivary tumour (15%). Ductal + myoepithelial cells with chondroid matrix is pleomorphic adenoma.
15	A	The smaller the gland involved, the more likely its tumour is malignant: parotid 80% benign, submandibular 50%, sublingual/minor 40%.
16	B	PA recurs if incompletely excised; carcinoma arising in it is carcinoma ex pleomorphic adenoma (malignant mixed tumour).
17	E	Table: PA 50% then Warthin 5% among benign; MEC 15% is the commonest malignant (acinic 6%, adenocarcinoma NOS 6%, adenoid cystic 4%).
18	A	Oral SCC: ulceration and induration; histology shows nests and islands of malignant keratinocytes invading connective tissue stroma.
19	D	Protracted irritation is weakly associated; tobacco is the best-established influence, and alcohol + tobacco interact strongly.
20	B	Listed causes: trauma, viral (mumps), bacterial (staph or strep after obstruction or dehydration) and autoimmune (Sjögren). HPV is an oral infection, not a listed cause of sialadenitis.

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Recalled and source questions are treated as evidence of what gets asked, not as a source of correct answers — verify against your lecture notes.